



NMHS POLICY

Recognising and Responding to Acute Deterioration (RRAD)

Scope (Staff)	All NMHS staff
Scope (Area)	All NMHS sites and services excluding community-based services or telehealth services provided by NMHS. <i>Note:</i> Community-based services and telehealth services provided by NMHS are required to possess local procedures or guidelines which outline processes to ensure early recognition and response to acute physiological and mental state deterioration which satisfy the applicable actions within the National Safety and Quality Healthcare Standards, taking into consideration of the local context.
Summary	This Policy outlines the minimum mandatory requirements for recognising and responding to acute physiological and mental state deterioration for all patients receiving care across NMHS sites and services, in accordance with the WA Health Mandatory Policy Recognising and Responding to Acute Deterioration Policy (MP 0171/22).
Approval	North Executive Team
Sponsor	Executive Director Consumer Experience and Clinical Excellence
Contact	Manager Clinical Governance and Performance
Version	1.2
Next Review Date	17/10/2026 (<i>approved extension from 16/4/2026</i>)
NSQHS Standards	Clinical Governance (Std 1) Partnering with Consumers (Std 2) Recognising and Responding to Acute Deterioration (Std 8)
ISD Number	1997
Status	CURRENT

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1. Aim

This Policy outlines the minimum mandatory requirements for recognising and responding to acute physiological and mental state deterioration for all patients receiving care across NMHS sites and services, in accordance with the [WA Health Mandatory Policy Recognising and Responding to Acute Deterioration Policy \(MP 0086/18\)](#).

This Policy enforces clinical guidance to meet requirements of the [Australian Commission on Safety and Quality in Health Care \(the Commission\), National Safety and Quality Health Service \(NSQHS\) Standards 'Standard 8: Recognising and Responding to Acute Deterioration'](#) – mandated through [MP 0134/20 National Safety and Quality Health Service Standards Accreditation Policy](#).

This Policy **does not** apply to community-based services or telehealth services provided by NMHS.

Community-based services and telehealth services provided by NMHS are required to possess local procedures or guidelines which outline processes to ensure early recognition and response to acute physiological and mental state deterioration which satisfy the applicable actions within the National Safety and Quality Healthcare Standards, taking into consideration of the local context.

2. Risk

Non-compliance with this Policy may result in a breach in mandatory policies, failure to meet the expected standards, harm to patients, their families and carers, and/or damage to the reputation and image of NMHS.

3. Definitions

Acute deterioration	Physiological or mental state changes that may indicate a decline of the patient's health status.
Community-based services	For the purpose of this policy, community-based services refer to services provided by NMHS in a community setting.
Deterioration in mental state	A negative change in a patient's mood or thinking, marked by a change in behaviour, cognitive function, perception or emotional state. Change can be gradual or acute; it can be observed by health professionals, reported by the patient themselves, or reported by their family or carers. Deterioration in mental state may relate to several predisposing or precipitating factors, including mental illness, psychological or existential stress, physiological changes, cognitive impairment (including delirium), intoxication, withdrawal from substances, and emotional response to social context and environment.
NMHS sites and services	Refer to Sir Charles Gairdner Osborne Park Health Care Group (SCGOPHCG), Women and Newborn Health Service (WNHS), and Mental Health Public Health Dental Services (MHPHDS).

4. Governance

The North Clinical Governance Committee will be responsible for:

- oversight and endorsement of Observation and Response Charts (ORC), including:
 - development and introduction of new charts and trigger tools
 - amendments to existing charts and trigger tools
 - requests to use alternative charts to those endorsed by NMHS
- providing direction on key NMHS wide decisions to meet the guiding principles of the:
 - [National Consensus Statement: Essential elements for recognising and responding to clinical deterioration](#)
 - [National Consensus Statement: Essential elements for recognising and responding to deterioration in a person's mental state](#)
 - [NSQHS Standards Advisory AS 19/01: Recognising and Responding to Acute Deterioration Standard: Recognising deterioration in a person's mental state](#)
- Oversight over the clinical pathways, tools and processes relating to:
 - Acute Anaphylaxis
 - Sepsis recognition and response
 - Deterioration in Mental State including Delirium
 - Goals of Patient Care (GoPC)
 - Advance Care Planning
 - Aishwarya's CARE Call

4.1 Site Governance

All NMHS sites and services must develop and implement a local procedure or clinical practice guideline that outlines processes to ensure early recognition and response to acute physiological and mental state deterioration, in line with this Policy, the [WA Health Mandatory Policy Recognising and Responding to Acute Deterioration Policy \(MP 0086/18\)](#) and [the NSQHS Standards Standard 8: Recognising and Responding to Acute Deterioration](#).

All NMHS sites and services must have a committee within their clinical governance structure responsible for the monitoring and evaluation of early recognition and response to acute physiological and mental state deterioration processes. The governance committee will be responsible for:

- implementation and evaluation of the NMHS endorsed ORC:
 - completion and submission of the ORC Request Form (see [Appendix 1](#)) to the [Executive Director, NMHS Consumer Experience and Clinical Excellence](#) when making a request for changes to an existing ORC or creation of a new ORC; the request will require tabling for endorsement at the NMHS Clinical Governance Committee.
 - oversight of arrangements for ordering and printing of amended and authorised ORC;
- development, endorsement and implementation of evidence based escalation protocols and procedures / clinical practice guidelines setting out the organisational response required to deal with different levels of abnormal physiological and mental state observations:

- consideration of site characteristics, available resources, and the potential need for the transfer of patients to another healthcare facility in the development of escalation protocols;
- development and endorsement of a protocol for patient / family / carer initiated escalation;
- establishment of a formal rapid response system (e.g. Medical Emergency Response (MER), Medical Emergency Team (MET), calling '000' for an ambulance) that includes the attendance of appropriately trained health professionals working within their clinical scope with the capability to address physiological and mental state deterioration, 24 hours per day, 7 days per week;
- ensuring clinical staff identify and adhere to GoPC and any form of advance care planning documents when managing or escalating care of a deteriorating patient;
- regular review of the effectiveness of the rapid response systems including identified key performance and outcome indicators;
- monitoring and ensuring local compliance with this Policy and associated site specific procedures / clinical practice guidelines.

5. Policy Principles

5.1 Monitoring for acute deterioration

All NMHS sites and services will utilise a NMHS-endorsed observation and response system (e.g. the utilisation of the single-parameter multi-tier ORC) that allows clinicians to monitor and document vital signs and any changes over time including:

- at time of initial assessment or admission;
- when a patient is experiencing, or at risk of experiencing, an episode of acute deterioration;
- prior to inter- or intra-hospital transfer.

The observation and response system must:

- be appropriate and modifiable for individual patient circumstances accounting for any form of advance care planning documents and GoPC forms;
- include provision for mental state observations (delirium, cognitive impairment, and psychological assessment) to recognise acute deterioration in mental state;
- include a prompt for clinicians to consider the possibility of sepsis.

Other assessment and monitoring tools in addition to the ORC may be required to alert acute deteriorations (e.g. Acute Anaphylaxis - refer to **section 5.2.1**, Sepsis – refer to **section 5.2.2**, Mental State Deterioration - refer to **section 5.2.3**).

5.1.1 Modifications

All NMHS sites and services must have the below specified within the local procedure:

- Who is authorised to make modifications to physiological parameters on the ORC;
- Who is authorised to make modifications to the frequency of mental state observations on locally endorsed mental state assessment tool (or equivalent)
- The valid period of each modification;
- The frequency the modification must be reviewed and by whom;
- When a valid modification is made that a clinical management plan must be documented on the ORC as well as the patient's healthcare record.

5.1.2 Advance Care Planning and/or GoPC

NMHS sites and services are responsible for ensuring that there are processes in place for the identification of GoPC forms and any form of advance care planning documents when managing or escalating care of a deteriorating patient. Refer to [NMHS Goals of Patient Care Policy](#) and the [WA Health Advance Care Planning website](#).

5.2 Escalation of acute deterioration

NMHS sites and services must develop and implement evidence-based escalation protocols that are appropriate to the local context to guide the escalation of care. The protocols must specify the organisational response required to respond to different levels of abnormal physiological and mental state observations, including:

- Clearly specifying responsibilities for managing care by the treating team and other support teams;
- Setting out clinical parameters for escalating care;
- Considering the potential need for the transfer of patients to another healthcare facility as part of the escalation;
- Ensuring that clinical staff identify and adhere to GoPC and any form of advance care planning documents when managing or escalating care of a deteriorating patient.

NMHS recognises the critical role of the patient, family and/or carers in patient care. NMHS sites and services must develop and implement a protocol for patient / family / carer-initiated escalation. Local processes must be in place to ensure these protocols are communicated with the patient, family and/or carers.

5.2.1 Acute Anaphylaxis

NMHS sites and services are required to have processes in place to ensure that known allergies or potential risks are documented in the clinical alerts in accordance with the [WA Health Clinical Alert MedAlert Policy \(MP 0053/17\)](#) and managed in accordance with the best evidence principles outlined in the [Commission's Acute Anaphylaxis Clinical Care Standard](#).

5.2.2 Sepsis

NMHS sites and services must develop and implement evidence-based sepsis pathways and sepsis management clinical practice guidelines that are appropriate to the local context, in accordance with the best evidence principles outlined in the [Commission's Sepsis Clinical Care Standard](#).

5.2.3 Mental State Deterioration

NMHS sites and services are required to have processes in place to ensure any mental state deterioration are identified and managed in accordance with principles outlined in the [National Consensus Statement: Essential elements for recognising and responding to deterioration in a person's mental state](#), including but are not limited to:

- Development and implementation of standardised mental state assessment tool (or equivalent)
- Development and implementation of evidence-based guidelines on the management of delirium in accordance with principles outlined in the [Commission's Delirium Clinical Care Standard](#).

5.3 Rapid response system for acute deterioration

NMHS sites and services must have a formal rapid response system to facilitate appropriate and timely response to acute deterioration.

The rapid response system (e.g. MER, MET, calling '000' for an ambulance) must include access to a rapid response team that consists of appropriately trained health professionals working within their clinical scope with the capability to address physiological and mental state deterioration, and operate 24 hours a day and 7 days a week.

Regular review and evaluation of the effectiveness of local rapid response system will be conducted by sites and services and reported annually to the NMHS Safety Quality Governance & Consumer Engagement.

5.3.1 Aishwarya CARE Call

Patients, their families or carers are often aware before clinical staff of subtle changes in their or their loved one's health, but may not feel comfortable, or know how to share their concerns with clinical staff. Coronial inquests have identified where concerns raised by the patient, family members or carers about the condition of the patient were not considered by clinical staff, leading to further deterioration with tragic outcomes.

[Aishwarya's CARE Call](#) provides patients, their families and carers with a pathway to request assistance when they feel that the healthcare team has not fully recognised the patient's changing health condition.

It supports established strategies to improve the recognition and response to clinical deterioration, including the use of NMHS ORC with defined escalation requirements and site-specific clinical escalation processes.

Refer to [NMHS Aishwarya's CARE Call Policy](#) for requirements on managing Aishwarya's CARE Call at NMHS. .

5.4 Staff education and training

Education on recognising and responding to acute deterioration will be provided to all clinical and non-clinical staff, within the scope of their role, on the commencement of their employment, and via regular ongoing education sessions. In addition, all staff should be made aware of relevant local policies, rapid response system and required documentation, and know how to activate the local rapid response system. Refer to [NMHS Employee Induction and On-boarding Policy](#) and [NMHS Mandatory Training Policy](#) for further information.

Each NMHS site and service will be responsible for ensuring ongoing education programs are regularly evaluated.

6. Compliance and Evaluation

Compliance to the Policy is mandatory.

Each NMHS site and service Clinical Governance Committee (or equivalent) will be responsible for developing local process to monitor compliance with this Policy and ensuring local compliance. NMHS Clinical Governance Committee may request sites/services to submit evidence of compliance in relation to the requirements of this Policy on an ad-hoc basis.

7. Document Control

Version	Amendment(s)	TRIM Ref	Published Date
1.0	Published version	D/23/39701	17/04/2023
1.1	Minor amendment: • 'NMHS RRAD Reference Group' changed to North Clinical Governance Committee and relevant sections updated to reflect this • Policy Sponsor position title changes to align with the name change of the directorate • Policy Contact changes to Manager Clinical Governance and Performance • Reference to NMHS Aishwarya's CARE Call Policy added • Policy compliance responsibility updated to sit with the North Clinical Governance Committee • Appendix 1 Request Form updated to include section to record decision by the North Clinical Governance Committee	D/24/97987	02/05/2024
1.2	Amendment to 'Compliance Monitoring and Evaluation' section.	D/24/150098	03/07/2024

Related policies, procedures and guidelines

[WA Health Mandatory Policy](#)

[WA Health Clinical Alert MedAlert Policy \(MP 0053/17\)](#)

[WA Health National Safety and Quality Health Service Standards Accreditation Policy \(MP 0134/20\)](#)

[WA Health Recognising and Responding to Acute Deterioration Policy \(MP 0171/22\)](#)

[NMHS Policy Documents](#)

[NMHS Aishwarya's CARE Call Policy](#)

[NMHS Employee Induction and On-boarding Policy](#)

[NMHS Goals of Patient Care Policy](#)

[NMHS Mandatory Training Policy](#)

[SCGOPHCG Policy Documents](#)

[SCGOPHCG Recognising and Responding to Acute Deterioration \(RRAD\) Procedure](#)

[SCGOPHCG Sepsis Management Clinical Practice Guideline](#)

[MHPHDS Policy Documents](#)

[Mental Health Recognising and Responding to Acute Deterioration \(RRAD\) Procedure \(in development\)](#)

[Dental Health Services Procedure Medical and Dental Emergencies](#)

WNHS Policy

Recognising and Responding to Acute Deterioration (Physiological and Mental Health)
Procedure (*in development*)

References

- [NSQHS Standards Recognising and Responding to Acute Deterioration Standard](#)
- [NSQHS Standards Advisory AS 19/01: Recognising and Responding to Acute Deterioration Standard: Recognising deterioration in a person's mental state](#)
- [Australian Commission on Safety and Quality in Health Care Acute Anaphylaxis Clinical Care Standard](#)
- [Australian Commission on Safety and Quality in Health Care Delirium Clinical Care Standard](#)
- [Australian Commission on Safety and Quality in Health Care Sepsis Clinical Care Standard](#)

Useful resources

- [WA Health Advance Care Planning website](#)
- [National Consensus Statement: Essential elements for recognising and responding to acute physiological deterioration \(third edition\)](#)
- [National Consensus Statement: Essential elements for recognising and responding to deterioration in a person's mental state](#)
- [NSQHS Standards user guide for health service organisations providing care for patients with cognitive impairment or at risk of delirium](#)
- [NMHS Sepsis Hub](#)

This document can be made available in alternative formats on request for a person with a disability.

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Appendix 1 NMHS Observation and Response Chart Request Form

The completion of this form is required to initiate a request for changes to an existing Observation and Response Chart (ORC) or creation of a ORC for use across NMHS.

Section A. Observation and Response Chart			
This request is for:		☐ Amendment to an existing ORC	
		☐ Creation of a new ORC	
Title of the ORC (if applicable)			
Scope of the ORC			
Sites and services			
Areas / Departments			
NMHS site and service making this request			
Site and service			
Committee Name			
Committee Chair			
NMHS staff submitting this request			
Name			
Position			
Section B. Assessment of the Request			
Risk Assessment			
Refer to NMHS Risk Assessment Tables			
☐ Low	☐ Medium	☐ High	☐ Extreme
Priority Rating			
☐ Low	☐ Medium	☐ High	☐ Urgent
Rationale for the new ORC / Amendment			
Amendments proposed to the existing ORC			
Section C. North Clinical Governance Committee (NCGC) Decision			
Date Tabled at NCGC			
NCGC Decision		☐ Endorsed	☐ Not Endorsed
Additional Comments			
NCGC Chair Name			
Position			
Signature			